

BIOSYNC CLINICAL LABORATORY REPORT

Patient: Nina Vasquez (DOB: 07/03/1998) | Date: March 26, 2026

Vital Signs & Biometric Readings

Heart Rate	99 BPM
Blood Pressure	122/78 mmHg
SpO2	90%
Temperature	101.1°F
Glucose	194 mg/dL
Weight	104 lbs
BMI	19.0
HRV	31 ms

Clinical Notes

Patient presents with second pulmonary exacerbation in 2 months. Increased productive cough, fever, and significant dyspnea reported. Ciprofloxacin allergy documented — avoided. Currently on Orkambi 200/125mg twice daily, Albuterol nebulizer (increased frequency), Creon 12,000 units, Azithromycin 500mg 3x/week, Vitamin ADEK daily, Dornase alfa daily. Heart rate 99 BPM — elevated, exacerbation state. SpO2 90% — critically low for outpatient management, approaching admission threshold. Temperature 101.1°F — fever indicating active infection. Glucose 194 mg/dL — CFRD decompensating with infection stress — endocrinology urgent referral. Weight 104 lbs — down 4 lbs from February — significant nutritional decline. HRV 31 ms severely depressed. Sputum culture from February: new organism — *Stenotrophomonas maltophilia* detected alongside *Pseudomonas* — complex antimicrobial management required.

Provider Notes

Second exacerbation in rapid succession — disease progression concern. Orkambi efficacy review — consider switching to Trikafta if eligible (genotype confirmation pending). IV antibiotics via port: TMP-SMX for *Stenotrophomonas* (no Cipro per allergy), Ceftazidime for *Pseudomonas*. Hospital admission strongly recommended — patient opts for home IV with daily nursing. SpO2 requires daily monitoring — admission threshold set at 88%. Insulin therapy restarted for CFRD. Nasogastric nutrition support discussed with patient. Urgent multidisciplinary CF team meeting scheduled. Follow-up in 5 days. Ordering Provider: Dr. Angela Holt, MD — Pulmonology/CF Center. Facility: BioSync Clinical Health Center